

Separate a risk factor from disease

Pressure alone cannot tell whether glaucomatous optic nerve damage is present.

Classify the open-angle picture

Category	Defining combination
Ocular hypertension	Raised IOP; no detectable nerve or field damage
POAG	Open angle; characteristic nerve/field damage; pressure raised at some stage
Normal tension glaucoma	Open angle and characteristic damage despite normal-range pressure
Secondary glaucoma	A recognisable contributing disorder must be identified

Interpret the whole assessment

Exclude angle closure and secondary causes before calling an eye ocular hypertension or primary open-angle disease. Compare disc structure with a reliable visual field rather than relying on an isolated cup ratio. A consistently normal measured pressure does not rule out glaucoma. Susceptibility of the nerve and factors affecting tonometry help explain why some eyes are damaged at lower pressures while others tolerate higher readings without detectable damage.

References Elijah para 3611; Elijah para 3618; Elijah para 3852; Elijah para 4017

Risk changes the follow-up plan

Ocular hypertension warrants risk assessment; established glaucoma requires protection against further functional loss.

Useful risk and measurement clues

OHT progression risk

Older age, higher IOP and low central corneal thickness add concern.

Normal tension context

Systemic hypotension and abnormal vasoregulation may be relevant history.

Pressure measurement

IOP fluctuates; note measurement timing and consider corneal thickness.

Keep monitoring purposeful

Treatment is not automatically required for every ocular hypertension patient; weigh risk and the absence or emergence of damage. In normal tension glaucoma, further IOP lowering remains a management strategy despite the normal-range starting pressure. Early POAG often gives no symptoms because central acuity can remain preserved. Follow-up should therefore assess the nerve and field as well as pressure, and review medication use that may raise or lower the reading.